

HEADQUARTERS, DEPARTMENT OF THE ARMY
Washington, DC

TC 4-02.1

TACTICAL COMBAT CASUALTY CARE (TCCC) GUIDELINES

Edge Brain Mesh Node: COMBAT CASUALTY CARE BRAIN

DISTRIBUTION: Approved for public release; distribution is unlimited.

Section 1. TCCC Guidelines / TC 4-02.1, MARCH

Massive hemorrhage. Apply a CoTCCC tourniquet high and tight on the limb, over the uniform, two to three inches above the wound. Tighten until bright red bleeding stops and the distal pulse is gone. Mark the time on the tourniquet. Do not loosen it.

Section 2. TCCC Guidelines / TC 4-02.1, MARCH

Airway. If unconscious, open the airway with a chin lift or insert a nasopharyngeal airway and place the casualty in the recovery position. Breathing. For a penetrating chest wound, apply a vented chest seal and monitor for tension pneumothorax.

Section 3. ATP 4-25.13, 9-Line MEDEVAC

To request evacuation, transmit the nine line MEDEVAC: Line 1 location, Line 2 your callsign and frequency, Line 3 number of patients by precedence, Line 4 special equipment, Line 5 patients by type litter or ambulatory, Line 6 security at pickup site, Line 7 method of marking, Line 8 patient nationality, Line 9 terrain or CBRN contamination.

Section 4. TCCC Guidelines, Circulation / TXA

Circulation. If the casualty is in hemorrhagic shock, establish IV or IO access and give tranexamic acid as soon as possible, ideally within one hour, but not after three hours from injury. Prevent hypothermia: remove wet gear and wrap the casualty in a warming blanket.

